Laboratory Investigation Report

Patient Name	: Mr. Hitesh Kumar	Centre	: 5245 - Integrated Kidney Solutions Pvt Ltd.
Age/Gender	: 32 Y 0 M 0 D /M	OP/IP No/UHID	: OP/OPCS53011/EKUI14183
MaxID/Lab ID	: ML05145739/4528102400657	Collection Date/Time	: 22/Oct/2024 09:12PM
Ref Doctor	: SELF	Reporting Date/Time	: 22/Oct/2024 10:55PM

Hematology

SIN No:B2B6173987

Test Name	Result	Unit	Bio Ref Interval
Malaria Antigen – P Vivax & P Falciparum*, EDTA			
Malaria Antigen Immumochromatography - pLDH & HRP2	Negative		Negative

Interpretation Rapid card test for malaria is a combo kit designed to test Plasmodium falciparum and Plasmodium vivax species of malaria. This is a combo kit coated with specific monoclonal antibodies against pLDH of the P. Vivax and HRP2 of the P. Falciparum. This kit can also detect the combined infection by these two species.

The result of this test needs to be corroborated with clinical features and other laboratory findings. Positive result with faint test line or false negative may be seen in low parasite density. Negative result can also be seen in prozone effect – i.e. very high antigen concentration compared to antibody concentration.

False positive result may be seen in acute Schistosomiasis.

Test may remain positive even after successful anti-malarial therapy and therefore should not be used for monitoring response to anti-malarial treatment.

Advice: “Peripheral smear for Malarial Parasite”

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CBC (Complete Blood Count)*, Whole Blood EDTA

Date	22/Oct/2024 09:12PM	Unit	Bio Ref Interval
Haemoglobin Modified cyanmethemoglobin	14.89	g/dl	13.0 - 17.0
Packed Cell, Volume Calculated	45.3	%	40-50
Total Leucocyte Count (TLC) Electrical Impedance	6.37	10~9/L	4.0-10.0
RBC Count Electrical Impedance	4.82	10~12/L	4.5-5.5
MCV Electrical Impedance	94.0	fL	83-101
MCH Calculated	30.9	pg	27-32
MCHC Calculated	32.9	g/dl	31.5-34.5
Platelet Count Electrical Impedance	**120**	10~9/L	150-410
MPV Calculated	**14.75**	fl	7.8-11.2
RDW Calculated	13.8	%	11.5-14.5
Differential Cell Count VCS / Light Microscopy			
Neutrophils	66	%	40-80
Lymphocytes	**19**	%	20-40
Monocytes	**14**	%	2-10
Eosinophils	01	%	1-6
Basophils	00	%	0-2
Absolute Leukocyte Count Calculated from TLC & DLC			
Absolute Neutrophil Count	4.2	10~9/L	2.0-7.0
Absolute Lymphocyte Count	1.2	10~9/L	1.0-3.0
Absolute Monocyte Count	0.89	10~9/L	0.2-1.0
Absolute Eosinophil Count	0.06	10~9/L	0.02-0.5

Kindly correlate with clinical findings

***** End Of Report *****

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Hematology

SIN No:B2B6173987

Sania

Dr. Sania Zahra Rizvi
MBBS,MD(Pathology)
DMC No.66798

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Serology

SIN No:B2B6173987

Test Name	Result	Unit	Bio Ref Interval
Widal Test (Slide)*, Serum			
Slide Agglutination			
Salmonella typhi, (O)	<1:80	Titre	<1:80
Slide Agglutination			
Salmonella typhi, (H)	<1:80	Titre	<1:160
Slide Agglutination			
Salmonella paratyphi (AH)	<1:80	Titre	<1:160
Slide Agglutination			
Salmonella paratyphi (BH)	<1:80	Titre	<1:160
Slide Agglutination			

Interpretation

1. This is slide agglutination test. Widal test by tube method is more specific and recommended test.
2. This is only screening test and definite diagnosis should not be based upon this single test.
3. 'H' titre > 1:160 and 'O' titre > 1:80 are positive however the treatment should be started based upon the clinical symptoms and other supplemental tests like blood culture and Widal tube method.

Advice:

1. First week of fever: Blood Culture.
2. Second week of fever: Widal tube test.

Kindly correlate with clinical findings

***** End Of Report *****

Sania

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SEROLOGY SPECIAL.

SIN No:B2B6173987

Test Name	Result	Unit	Bio Ref Interval
Dengue NS 1 Antigen Test			
Dengue NS 1 Antigen CLIA	0.12	Index	

Ref. Range

Negative ≤ 0.9
Equivocal 0.9 - 1.1
Positive > 1.1

Comment :

- The detection of NS1 antigen has been described as an alternative method for early diagnosis of dengue virus infection.
- NS1 antigen was found circulating from the first day and up to 9 days after the onset of fever, with comparable levels observed in primary and secondary infections.
- A negative results does not preclude the possibility of early dengue virus infection.

Note: Recommended test is NS1 Antigen by ELISA in the first 5 days of fever. After 7-10 days of fever, the recommended test is Dengue fever antibodies IgG & IgM by ELISA

Kindly correlate with clinical findings

***** End Of Report *****

Dr.Poonam.S. Das, M.D.
Principal Director-
Max Lab & Blood Bank Services

Dr. Bansidhar Tarai, M.D
Director, Microbiology & Molecular Diagnostics

Dr. Sonu Kumari Aggrawal, MD
Consultant Microbiology

Dr Nidhi Malik, MD
Consultant Microbiology

Test Performed at :910 - Max Hospital - Saket M S S H, Press Enclave Road, Mandir Marg, Saket, New Delhi, Delhi 110017
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